

CCP Pipeline Run: Jane Doe

Output of running `ccp.consort + ccp/medical-care.consort` against `ccp/samples/jane-doe-health-record.txt`, with `$ show intermediate stages` set. Generated by Claude (Sonnet 5) acting as the interpreting model, in a Claude Code conversation, on 2026-09-12.

Fictional demonstration only -- synthetic patient, not real medical advice. "Jane Doe" is the standard placeholder name; this is not a real person or a real medical record.

Intermediate Stages

OBSERVE

58F, 4-day history of feeling unwell. Reported: non-productive cough (worsening x2 days), subjective fever ("feeling hot," no objective home reading), sore throat, nasal congestion, fatigue/malaise, mild exertional dyspnea. No chest pain, no GI symptoms. Vitals: Temp 100.8F, HR 92, RR 20, BP 132/84, SpO2 95% RA. Labs: WBC 11.2 (mildly elevated), CRP 42 (elevated), glucose 168 random (elevated), otherwise normal chemistry/CBC. Urinalysis unremarkable except trace glucose. History: type 2 diabetes (oral agents, unspecified), recurrent past winter respiratory infections (patient-reported), one prior pneumonia (~3 years ago, outpatient-treated). No known drug allergies.

ASSESS

Mild tachycardia and tachypnea, low-grade fever, SpO2 95% -- none individually or combined meet emergency/life-threatening criteria on the data available; urgent escalation is **not** indicated at this time. Diabetes and prior pneumonia are relevant risk factors lowering the threshold for closer follow-up. Missing: chest exam/auscultation findings, imaging, pathogen testing, confirmed temperature, current glycemic control data (HbA1c, medication list).

INVESTIGATE

Available evidence (mild leukocytosis + elevated CRP) supports an active inflammatory/infectious process but doesn't distinguish viral from bacterial or upper from lower respiratory tract. Chest imaging, auscultation, and pathogen testing have **not** been performed -- treated as not-yet-available, not as negative.

DIAGNOSE

Findings are consistent with an acute respiratory infection of indeterminate type. Differential: (a) viral upper respiratory infection / acute bronchitis -- most consistent with symptom pattern and only mild marker elevation; (b) early/mild community-acquired pneumonia -- given prior pneumonia history, diabetes, and mild dyspnea, cannot be excluded without imaging. No chest pain and SpO2 95% neither confirm nor rule out (b). **This is a differential, not a confirmed diagnosis** -- current evidence does not support a definitive label.

PLAN

Confirm or exclude a lower respiratory process before committing to antibiotics; manage symptoms in the interim; monitor glycemic control given illness-associated hyperglycemia risk in a diabetic patient.

PRESCRIBE (recommendations, not yet authorized/administered)

Chest X-ray; consider respiratory pathogen panel; symptomatic care (antipyretic/analgesic, hydration, rest); increased home glucose-monitoring frequency; explicit return precautions (worsening dyspnea, chest pain, confusion, persistent high fever, inability to maintain hydration/glucose control). **No antibiotics prescribed** -- evidence doesn't yet support a bacterial process.

TREAT

No intervention has actually been performed within this record beyond the recommendations above -- imaging and symptomatic measures are the intended next actions, execution pending outside this record.

MONITOR

Not yet applicable -- no treatment-response or imaging data exists yet. Would track: symptom trajectory, vital trends, glucose control, and pending results.

REASSESS

Cannot be performed against an expected response yet -- pending diagnostic workup.

ADAPT

Appropriate next action is to **loop back to INVESTIGATE** once imaging/testing results return, rather than proceeding further on current evidence -- this is the closed-loop point (spec 2.9): re-invoking this pipeline from `investigate` onward is an external orchestrator decision once new evidence exists, not something resolved within this pass.

FOLLOWUP

Re-evaluate after imaging/testing complete. Escalation criteria: worsening dyspnea, sustained/rising fever, chest pain, confusion, or inability to manage glucose/hydration at home. **Episode remains open**, not concluded.

Final Clinical Coordination Record

Current clinical state: 58F with a 4-day acute respiratory illness (cough, sore throat, congestion, subjective fever, mild dyspnea), stable vitals, no hypoxia or emergency findings.

Evidence: Mild leukocytosis, elevated CRP, elevated random glucose; otherwise normal labs/urinalysis; no imaging or exam findings available.

Diagnosis: Differential of viral URI/acute bronchitis vs. early mild pneumonia -- unresolved pending imaging.

Treatment plan: Chest X-ray, possible pathogen testing, symptomatic care, glucose monitoring; antibiotics deferred.

Interventions/response: None yet executed or observed.

Unresolved uncertainty: Etiology and whether a lower respiratory process (pneumonia) is present.

Required follow-up: Re-assessment once imaging/testing results are available; explicit escalation criteria given; episode open.